



MRI Report – Head and Neck (W/ & W/O Contrast)

Department: Diagnostic Imaging – MRI
Facility: NorthStar Medical Imaging (MRI Suite)
Exam: MRI Head and Neck with and without Contrast
Date of Exam: 2021-12-15
Comparison: Ultrasound neck 2021-10-18
Radiologist: Network Attending (synthetic signature)

****Indication:**** Persistent cervical lymphadenopathy. Prior ultrasound demonstrated atypical cortical thickening. Evaluate for deep nodal disease, extranodal extension, or occult head/neck primary lesion.

****Technique:**** MRI head and neck performed with and without gadolinium contrast. Multiplanar T1/T2 sequences obtained with fat suppression. Post■contrast imaging acquired through the skull base to thoracic inlet. Study quality adequate.

****Findings – Lymph Nodes:**** Scattered lymph nodes are seen bilaterally in the cervical chains and within/adjacent to the parotid regions. Nodes retain benign-appearing morphology without central necrosis, matted appearance, or abnormal capsular disruption. The largest measured lymph node is in the right jugulodigastric region, approximately ****18 mm**** (short axis). No pathologic retropharyngeal nodes identified.

****Findings – Mucosal Spaces:**** Nasopharynx, oropharynx, hypopharynx, and laryngeal regions show no discrete enhancing mass within the imaged field. Parapharyngeal spaces are preserved. No abnormal asymmetric signal in the tongue base.

****Salivary Glands/Thyroid:**** Parotid and submandibular glands show no focal mass. Visualized thyroid parenchyma appears unremarkable.

****Osseous/Other:**** No suspicious marrow signal in imaged mandible/cervical spine. Visualized intracranial structures show no acute abnormality on limited sequences.

****Impression:**** (1) ****No suspicious cervical lymphadenopathy**** on MRI; scattered nodes appear benign. (2) No primary head/neck mass identified. (3) Recommend continued clinical follow■up and correlation with ultrasound trend; tissue diagnosis may still be required if progressive enlargement persists.



Addendum / Reporting Notes

This document uses standardized institutional templates to emulate real-world medical record formatting for prototype testing. All names, identifiers, and institutional details are fictional. Clinical content is synthesized from the provided longitudinal summary.

If results appear internally inconsistent across documents, this is intentional to mimic typical real-world fragmentation. When discrepancies arise, the recommended approach is to reconcile using original source dates and the most recent verified medication/problem lists.